

PATIENT HISTORY:

Not provided. LMP: Not provided.
PRE-OP DIAGNOSIS: Left breast carcinoma.
POST-OP DIAGNOSIS: Same.

PROCEDURE: Left segmental mastectomy needle localization, sentinel node biopsy left axilla.

COMMENT:

Foci of malignant clusters and glands surrounded by retraction type spaces are identified, and are stained for endothelial markers (D2-40 and CD31) for a more definitive interpretation. The results will be reported in an addendum.
ER, PR and Her-2/Neu were performed on the core biopsy and are reported as positive for receptors and negative for Her-2/Neu.

Pathologic stage: T1c N0 MX.

ADDENDA:

Addendum

Immunostain for endothelial markers (lymphatic and vascular) highlight the endothelial cells and confirms the absence of lymphovascular space involvement by tumor.

MICROSCOPIC:

Utilizing formalin-fixed (8-96 hour range), paraffin embedded tissue, immunohistochemistry is performed with the following selected antibodies and designated antibody clone(s), directed against the following antigenic target(s), with adequate positive and negative internal and external controls. Antibodies are optimized appropriate for fixation times.

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ANTIBODY	CLONE	TARGET ANTIGEN	VENDOR
D2-40	D2-40	Lymphatic Endothelium	Dako
CD 31	JC70A	endothelium	Dako

EE/shm

The following statement applies to all immunohistochemistry, insitu hybridization (ISH & FISH), molecular anatomic pathology, and immunofluorescence testing:

The testing was developed and its performance characteristics determined by the required by the CLIA '88 regulations. The testing has not been cleared or approved for the specific use by the U.S. Food and Drug Administration, but the FDA has determined such approval is not necessary for clinical use.

This laboratory is certified under the Clinical Laboratory Improvement Amendments of 1988 ("CLIA") as qualified to perform high-complexity clinical testing. Pursuant to the requirements of CLIA, ASR's used in this laboratory have been established and verified for accuracy and precision. Additional information about this type of test is available upon request.

CASE SYNOPSIS:

SYNOPTIC - PRIMARY INVASIVE CARCINOMA OF BREAST

LATERALITY: Left
PROCEDURE: Segmental
LOCATION: Upper inner quadrant
SIZE OF TUMOR: Maximum dimension invasive component: 1.6 cm
MULTICENTRICITY/MULTIFOCALITY OF INVASIVE FOCI: Yes
TUMOR AGGREGATE SIZE: Sum of the sizes of multiple invasive tumors: 1.9 cm
TUMOR TYPE (invasive component): Ductal adenocarcinoma, NOS

NOTTINGHAM SCORE:
Nuclear grade: 2
Tubule formation: 2
Mitotic activity score: 1
Total Nottingham score: 5
Nottingham grade (1, 2, 3): 1

ANGIOLYMPHATIC INVASION: No
DERMAL LYMPHATIC INVASION: Not applicable
CALCIFICATION: Yes, benign zones
SURGICAL MARGINS INVOLVED BY INVASIVE COMPONENT: No

SURG MARGINS INVOLVED BY IN SITU COMPONENT: No

LYMPH NODES POSITIVE: 0
LYMPH NODES EXAMINED: 2
METHOD(S) OF LYMPH NODE EXAMINATION: H/E stain

T STAGE, PATHOLOGIC: pT1c
N STAGE, PATHOLOGIC: pN0
M STAGE, PATHOLOGIC: pMX
ESTROGEN RECEPTORS: previously performed
PROGESTERONE RECEPTORS: previously performed
HER2/NEU: zero or 1+

1CD-0-3
carcinoma, infiltrating duct, NOS
8.500/3
Site: breast, NOS CSO.9

hw
10/15/11

Criteria	Yes	No
Diagnosis Discrepancy		
Primary Tumor Site Discrepancy		
HIPAA Discrepancy		
Prior Malignancy History		
Dual/Synchronous Primary Noted		
Case is (circle):	QUALIFIED	DISQUALIFIED
Reviewer Initials	hw	10/15/11

Criteria	Yes	No
Diagnosis Discrepancy		
Primary Tumor Site Discrepancy		
HIPAA Discrepancy		
Prior Malignancy History		
Dual/Synchronous Primary Noted		
Case is (circle): <u>QUALIFIED</u> / DISQUALIFIED		
Reviewer Initials: <u>MB</u> Date Reviewed: <u>4/24/11</u>		

FINAL DIAGNOSIS:

PART 1: BREAST, LEFT, 10 O'CLOCK, NEEDLE LOCALIZED SEGMENTAL MASTECTOMY -

- A. INVASIVE DUCTAL CARCINOMA, TWO FOCI, 1.6 CM (GROSS) AND 0.3 CM (MICROSCOPIC), NOTTINGHAM GRADE 1 (COMBINED NOTTINGHAM SCORE 5/9: TUBULE FORMATION -2/3, NUCLEAR GRADE- 2/3, MITOTIC ACTIVITY- 1/3) WITH LOBULAR FEATURES.
- B. DUCTAL CARCINOMA IN SITU IS NOT IDENTIFIED.
- C. MARGINS FREE, CLOSEST ANTERIOR WITHIN 0.2 CM FROM THE LARGEST TUMOR FOCUS, SECOND CLOSEST LATERAL WITHIN 0.5 CM FROM THE SMALLER TUMOR FOCUS.
- D. LYMPHOVASCULAR INVASION (SEE COMMENT).
- E. ATYPICAL DUCTAL EPITHELIAL HYPERPLASIA.
- F. DUCTAL EPITHELIAL HYPERPLASIA.
- G. COLUMNAR CELL CHANGES AND HYPERPLASIA WITH ASSOCIATED MICROCALCIFICATIONS.
- H. FIBROADENOMATOID CHANGES.
- I. PSEUDOANGIOMATOUS STROMAL HYPERPLASIA (PASH).
- J. CAPILLARY HEMANGIOMA.
- K. CAUTERY ARTIFACT.
- L. BIOPSY SITE CHANGES.
- M. PATHOLOGIC STAGE (see comment).

PART 2: NEW LATERAL MARGIN, EXCISION -

- A. NEGATIVE FOR TUMOR.
- B. FIBROADENOMATOID CHANGES.
- C. FIBROCYSTIC CHANGES.
- D. COLUMNAR CELL CHANGES.
- E. DUCTAL EPITHELIAL HYPERPLASIA, MILD.
- F. ADENOSIS.
- G. PSEUDOANGIOMATOUS STROMAL HYPERPLASIA (PASH).
- H. CAUTERY ARTIFACT.

PART 3: SENTINEL LYMPH NODE #1, LEFT AXILLA, BIOPSY -

ONE LYMPH NODE, NEGATIVE FOR METASTATIC CARCINOMA (0/1).

PART 4: SENTINEL LYMPH NODE #2, LEFT AXILLA, BIOPSY -

ONE LYMPH NODE, NEGATIVE FOR METASTATIC CARCINOMA (0/1).

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